

# Case Management Start of Care Assessment

CCG: TC-1044 (TC)

**MCG Health**  
Transitions of Care  
28th Edition

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## Assessment

### Language and Culture

**Q1. Patient:** Do you speak English?

- Yes
- No [Q2] [Q3]
- Unknown [Q2] [Q3]

**Q2. Case Manager:** What language does the patient speak?

- Arabic [B1]
- Armenian [B1]
- Cambodian [B1]
- Chinese [B1]
- Croatian [B1]
- Czech [B1]
- Danish [B1]
- Dutch [B1]
- Filipino (Tagalog) [B1]
- Finnish [B1]
- Formosan [B1]
- French [B1]
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- Greek [B1]
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- Hmong [B1]
- Hungarian [B1]
- Italian [B1]
- Japanese [B1]
- Korean [B1]
- Lithuanian [B1]
- Navaho [B1]
- Norwegian [B1]
- Persian [B1]
- Polish [B1]
- Portuguese [B1]

- Punjabi [B1]
- Romanian [B1]
- Russian [B1]
- Serbo-Croatian [B1]
- Slovak [B1]
- Spanish [B1]
- Swedish [B1]
- Thai (Laotian) [B1]
- Turkish [B1]
- Ukrainian [B1]
- Vietnamese [B1]
- Yiddish [B1]
- Other, specify: \_\_\_\_\_ [B1]
- Unknown [B1]

**Q3. Patient:** Is there anyone available who can translate?

- Yes, the patient's family or caregiver is available and speaks and understands English. [Q4] [Q5] [B1]
- Yes, the patient's family or caregiver speaks and understands English, but arrangements must be made for them to be available. [Q4] [Q5] [B1]
- No [Q6] [B1] [B2] [P1]
- Unknown [Q6] [B1] [B2] [P1]

**Q4. Caregiver:** Is the patient OK with you speaking on their behalf?

- Yes [Q5]
- No [Q6] [P1]
- Unknown [Q6] [P1]

**Q5. Patient:** Are there problems with reading or writing?

- Yes [B1] [P1] Health Literacy  TC
- No
- Unknown [P1] Health Literacy  TC

**Q6. Patient:** Is it OK to use a translation service?

- Yes
- No [B2] [B1] [P1]
- Unknown [B2] [B1] [P1]
- Not applicable

**Q7. Patient:** Were you born and raised in this country?

- Yes
- No [Q8] [Q9] [Q10] [Q11]
- Unknown

**Q8. Patient:** What country were you born and raised in?

- Specify: \_\_\_\_\_
- Unknown

**Q9. Patient:** When did you come to the United States?

- Specify: \_\_\_\_\_
- Unknown

**Q10. Patient:** How long have you lived in this country?

- Specify: \_\_\_\_\_
- Unknown

**Q11. Patient:** Are you a US citizen?

- Yes
- No
- Unknown

**Q12. Patient:** What is your ethnic background? (for example, Hispanic, Asian)

- Specify: \_\_\_\_\_
- Unknown

**Q13. Patient:** What is your religion? (for example, Catholic, Mormon, Jehovah's Witness)

- Specify: \_\_\_\_\_
- I don't practice any religion.
- Unknown

**Q14. Patient:** Are there medical treatments you do not want or that you are not supposed to get?

- Yes, specify: \_\_\_\_\_ [B3] [P2]
- No
- Unknown [P2]

**Q15. Patient:** Are there medicines you do not want or that you are not supposed to take?

- Yes, specify: \_\_\_\_\_ [B3] [P2]
- No
- Unknown [P2]

**Q16. Patient:** Are there any foods or drinks that you do not want or that you are not supposed to have?

- Yes, specify: \_\_\_\_\_ [B3] [P2]
- No
- Unknown [P2]

**Q17. Patient:** Are there any foods or drinks that you want or that you are encouraged to have?

- Yes, specify: \_\_\_\_\_ [B3] [P2]
- No
- Unknown [P2]

**Q18. Patient:** Are there any other considerations that you want to be included in your care plan?

- Yes, specify: \_\_\_\_\_ [B3] [P2]
- No
- Unknown [P2]

## Hearing and Vision

**Q1. Case Manager:** Can the patient hear well enough to engage in activities?

- Yes
- No [B4] [P3]
- Unknown [B4] [P3]

**Q2. Patient:** Do you have a hearing problem?

- Yes [Q3] [Q5] [B4] [P4]
- No [Q4]
- Unknown [Q4] [P3]

**Q3. Patient:** What do you use to help your hearing or make up for your hearing loss? (Check all that apply.)

- Hearing aids
- Sign language
- Sign language interpreter
- Telecommunications relay service (TRS) (eg, TTY or TDD)
- Writing notes
- Other, specify: \_\_\_\_\_
- Unknown [P4]
- Nothing [P4]
- Not applicable

**Q4. Patient:** Do your family members or friends think you have a problem hearing?

- Yes [Q5] [B4] [P4]
- No
- Unknown [P3]

**Q5. Patient:** Have you seen an otolaryngologist (a specialist hearing doctor) about your hearing problem?

- Yes
- No [P3]
- Unknown [P3]
- Not applicable

**Q6. Patient:** Do you have problems with your vision?

- Yes, specify: \_\_\_\_\_ [Q7] [Q8] [B5] [P5]
- No
- Unknown [P6]

**Q7. Patient:** Have you seen an ophthalmologist (a specialist eye doctor) about your vision problem?

- Yes
- No [P6]
- Unknown [P6]

**Q8. Patient:** What do you use to improve your vision or make up for your vision loss? (Check all that apply.)

- Braille [P5]
- Computer software [P5]
- Glasses or contact lenses [P5]
- Global Positioning System (GPS) for the visually impaired [P5]
- Magnifying lenses [P5]
- Sighted companion [P5]
- White cane or mobility training [P5]
- Other, specify: \_\_\_\_\_ [P5]
- Unknown [P5]
- Nothing [P6]
- Not applicable

**Q9. Patient:** Do you have trouble seeing to do daily activities? (Check all that apply.)

- Ambulation [B5] [P5]
- Cooking [B5] [P5]
- Dressing [B5] [P5]
- Driving [B5] [P5]
- In low light [B5] [P5]
- Reading [B5] [P5]
- Shaving [B5] [P5]
- Shopping [B5] [P5]
- Watching TV [B5] [P5]
- Other, specify: \_\_\_\_\_ [B5] [P5]
- Unknown [P6]
- No

## History and Health Status

**Q1. Patient: Major Diagnosis:** Please tell me about your current health status. What health problems do you have and when were they diagnosed?

- Anxiety, date diagnosed: \_\_\_\_\_
- Arthritis, date diagnosed: \_\_\_\_\_
- Asthma, date diagnosed: \_\_\_\_\_
- Atrial fibrillation, date diagnosed: \_\_\_\_\_
- Attention-deficit hyperactivity disorder (ADHD), date diagnosed: \_\_\_\_\_
- Autism spectrum disorder, date diagnosed: \_\_\_\_\_
- Back pain, date diagnosed: \_\_\_\_\_
- Bipolar disorder, date diagnosed: \_\_\_\_\_
- Blood or clotting disorder, date diagnosed: \_\_\_\_\_
- Cancer, date diagnosed: \_\_\_\_\_
- Chronic obstructive pulmonary disease (COPD), date diagnosed: \_\_\_\_\_
- Coronary artery disease, date diagnosed: \_\_\_\_\_
- Cystic fibrosis, date diagnosed: \_\_\_\_\_
- Dementia, date diagnosed: \_\_\_\_\_
- Depression, date diagnosed: \_\_\_\_\_
- Diabetes, date diagnosed: \_\_\_\_\_
- Heart failure, date diagnosed: \_\_\_\_\_
- High blood pressure (hypertension), date diagnosed: \_\_\_\_\_
- High cholesterol (hypercholesterolemia), date diagnosed: \_\_\_\_\_
- High-risk pregnancy, date diagnosed: \_\_\_\_\_
- HIV/AIDS, date diagnosed: \_\_\_\_\_

- Inflammatory bowel disease, date diagnosed: \_\_\_\_\_
- Kidney disease, date diagnosed: \_\_\_\_\_
- Liver disease, date diagnosed: \_\_\_\_\_
- Multiple sclerosis, date diagnosed: \_\_\_\_\_
- Obesity, date diagnosed: \_\_\_\_\_
- Osteoporosis, date diagnosed: \_\_\_\_\_
- Parkinson disease, date diagnosed: \_\_\_\_\_
- Pregnancy, date diagnosed: \_\_\_\_\_
- Schizophrenia, date diagnosed: \_\_\_\_\_
- Seizure disorder, date diagnosed: \_\_\_\_\_
- Sickle cell disease, date diagnosed: \_\_\_\_\_
- Spinal cord injury, date diagnosed: \_\_\_\_\_
- Stroke, date diagnosed: \_\_\_\_\_
- Substance use disorder, date diagnosed: \_\_\_\_\_
- Traumatic brain injury, date diagnosed: \_\_\_\_\_
- Other, specify: \_\_\_\_\_
- Unknown

**Q2. Patient: Surgical History:** What surgeries have you had and when did you have them? (Check all that apply.)

- Cardiac procedure, specify: \_\_\_\_\_
- Cosmetic procedure, specify: \_\_\_\_\_
- Gastrointestinal procedure, specify: \_\_\_\_\_
- General surgical procedure, specify: \_\_\_\_\_
- Gynecologic, specify: \_\_\_\_\_
- Head or neck surgical procedure, specify: \_\_\_\_\_
- Neurosurgical procedure, specify: \_\_\_\_\_
- Obstetric procedure, specify: \_\_\_\_\_
- Ophthalmologic procedure, specify: \_\_\_\_\_
- Orthopedic procedure, specify: \_\_\_\_\_
- Pulmonary procedure, specify: \_\_\_\_\_
- Renal procedure, specify: \_\_\_\_\_
- Skin or wound procedure, specify: \_\_\_\_\_
- Transplant, specify type and when: \_\_\_\_\_
- Urologic procedure, specify: \_\_\_\_\_
- Other, specify: \_\_\_\_\_
- None
- Unknown

**Q3. Patient: Treatment History:** Please describe your treatment for your main health problem. I'm interested in medicines, hospital stays, procedures, and other treatments.

- Treatment history: \_\_\_\_\_

**Q4. Patient:** What are the names, doses, routes, frequencies, and purpose for all the medicines you take? (Include nonprescription medicines.)

- No medicine taken
- Medicine, dose, route, frequency, purpose: \_\_\_\_\_
- Medicine, dose, route, frequency, purpose: \_\_\_\_\_
- Medicine, dose, route, frequency, purpose: \_\_\_\_\_
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- Medicine, dose, route, frequency, purpose: \_\_\_\_\_
- Medicine, dose, route, frequency, purpose: \_\_\_\_\_
- Unknown [P7]

**Q5. Patient:** Do you have fewer than 2 refills for any of your medicine? (Look at the prescription label for the information on "number of refills before" date.)

- Yes [P8]
- No
- Unknown [P7]
- Not applicable

**Q6. Patient: Hospital Inpatient Stays:** Have you ever stayed overnight in a hospital? If so, when and for what reason?

- Hospital inpatient stays: \_\_\_\_\_ [Q7] Hospitalization Risk Evaluation [TC](#)

**Q7. Patient:** Have you been to the emergency room since your last hospitalization?

- Yes Hospitalization Risk Evaluation [TC](#)
- No
- Unknown

**Q8. Patient:** Do you seek healthcare from anyone other than your doctor (eg, healers, herbalists, or acupuncturists)?

- Yes, specify: \_\_\_\_\_ [P9]
- No
- Unknown [P9]

**Q9. Patient:** Do you have any medical allergies (such as to medicines or latex)?

- Yes, specify: \_\_\_\_\_
- No
- None known
- Unknown

**Q10. Patient:** How tall are you?

- Height: \_\_\_\_\_

**Q11. Patient:** How much do you weigh?

- Weight: \_\_\_\_\_ Nutritional Status, Adult [TC](#)

**Q12. Patient:** What vaccinations have you had? (Check all that apply.) **QM**

- Coronavirus disease 2019 (COVID-19) [Q13]
- *Haemophilus influenzae* type b (Hib) [Q13] [Q14]
- Hepatitis A [Q13]
- Hepatitis B [Q13]
- Human papillomavirus (HPV) [Q13]
- Influenza [Q13] [Q14]
- Measles, mumps, rubella (MMR) [Q13]
- Meningococcal 4-valent conjugate (MenACWY) or polysaccharide (MPSV4) [Q13]
- Meningococcal B (MenB) [Q13]
- Pneumococcal 13-valent conjugate (PCV13) [Q13] [Q15]
- Pneumococcal 15-valent conjugate (PCV15) [Q13] [Q15]
- Pneumococcal 20-valent conjugate (PCV20) [Q13] [Q15]
- Pneumococcal 23-valent polysaccharide (PPSV23) [Q13] [Q15]
- Polio (inactivated) [Q13]
- Respiratory syncytial virus (RSV) [Q13]
- Rotavirus [Q13]
- Tetanus booster [Q13]
- Tetanus, diphtheria, pertussis (Td/Tdap) [Q13]
- Varicella [Q13]
- Zoster [Q13]
- Other, specify: \_\_\_\_\_ [Q13]
- None, because: \_\_\_\_\_ [P10]

- Unknown [P10]

**Q13. Patient:** Are you up to date with the vaccinations you need (for age, booster status, or medical condition)? **QM**

- Yes
- No, because: \_\_\_\_\_ [Q16] [P10]
- Unknown [Q16] [P10]

**Q14. Patient:** When was your last flu shot? **QM**

- In the past year
- More than a year ago [Q16] [P10]
- Never [Q16] [P10]
- Unknown [Q16] [P10]

**Q15. Patient:** When was your last pneumonia shot?

- In the last year
- 2 to 4 years ago
- 4 to 5 years ago [Q16] [P10]
- More than 5 years ago [Q16] [P10]
- Never [Q16] [P10]
- Unknown [Q16]

**Q16. Patient:** Do you have an appointment scheduled to get caught up on your vaccinations?

- Yes
- No [P10]
- Unknown [P10]

## Physical and Cognitive Status

**Q1. Patient:** Do you get regular exercise?

- Yes Exercise  TC
- No
- Unknown

**Q2. Patient:** Does your health limit your activities of daily living, such as bathing, dressing, eating, or moving around?

- Yes Activities of Daily Living  TC Safety  TC
- No
- Unknown Activities of Daily Living  TC Safety  TC

**Q3. Patient:** Does your health limit your instrumental activities of daily living, such as doing housework, taking your medicine, managing money, getting to and from places, or shopping?

- Yes Instrumental Activities of Daily Living  TC
- No
- Unknown Instrumental Activities of Daily Living  TC

**Q4. Patient:** Do you use any devices to assist you for care or safety?

- Yes Assistive Devices  TC Safety  TC
- No
- Unknown Assistive Devices  TC Safety  TC

**Q5. Patient:** Do you sometimes have problems thinking or remembering? If so, what problems do you have? (Check all that apply.) **QM**

- Activities of daily living tasks (eg, bathing, oral hygiene) [Q6] [P11]
- Appointment forgetfulness [Q6] [P11]
- Concentrating during conversations [Q6] [P11]
- Date (ie, time, month, year) [Q6] [P11]
- Decisiveness [Q6] [P11]
- Impulsive [Q6] [P11]
- Instrumental activities of daily living tasks (eg, grocery shopping, paying bills) [Q6] [P11]
- Location (ie, getting lost) [Q6] [P11]
- Medicine forgetfulness [Q6] [P11]
- Names or people [Q6] [P11]

- Past events, recent [Q6] [P11]
- Problem solving [Q6] [P11]
- Safety steps (eg, locking doors, turning off stove) [Q6] [P11]
- Train of thought [Q6] [P11]
- No
- Unknown [Q6] [P11] Behavioral Health and Cognition Evaluation with Caregiver [TC](#)

**Q6. Patient:** How does your current status of thinking or remembering compare with that of 6 months ago?

- About the same
- Better
- Worse [B6] [P11] Behavioral Health and Cognition Evaluation with Caregiver [TC](#) Preliminary Depression Screening [TC](#)
- Unknown [B6] [P11] Behavioral Health and Cognition Evaluation with Caregiver [TC](#) Preliminary Depression Screening [TC](#)

**Q7. Patient:** Do you have problems with day-to-day life because of trouble thinking or remembering? **QM**

- Yes [Q8] [B6] [P11] Behavioral Health and Cognition Evaluation with Caregiver [TC](#) Preliminary Depression Screening [TC](#)
- No
- Unknown [Q8] [B6] [P11] Behavioral Health and Cognition Evaluation with Caregiver [TC](#) Preliminary Depression Screening [TC](#)

**Q8. Patient:** Is there someone who helps you with your self-care and daily responsibilities? If so, who? **QM**

- Yes, specify: \_\_\_\_\_ Caregiver Resources Evaluation [TC](#)
- No [B6] [P11]
- Unknown [B6] [P11] Behavioral Health and Cognition Evaluation with Caregiver [TC](#) Preliminary Depression Screening [TC](#)

## Psychosocial Assessment

**Q1. Patient:** What is your housing situation today?(1) **QM**

- Do not have housing (eg, staying with others, hotel, shelter), specify: \_\_\_\_\_ [B7] [P12]  
[P13] Housing Insecurity [TC](#)
- Have own housing (eg, house, apartment), but afraid of losing it in the future [P12] [P13] Housing Insecurity [TC](#)
- Have own housing
- Other, specify: \_\_\_\_\_ [P12] [P13]
- Unknown

**Q2. Patient:** Do you have any problems with the place you live? (Check all that apply.) **QM**

- Appliances not working or unavailable (eg, stove, refrigerator) [P12] [P13] Housing Insecurity [TC](#)
- Bug infestation [P12] [P13] Housing Insecurity [TC](#)
- Electricity not working or unavailable [P12] [P13] Housing Insecurity [TC](#)
- Heating insufficient [P12] [P13] Housing Insecurity [TC](#)
- Lead paint or pipes [P12] [P13] Housing Insecurity [TC](#)
- Mold [P12] [P13] Housing Insecurity [TC](#)
- Smoke detectors not working or unavailable [P12] [P13] Housing Insecurity [TC](#)
- Water leaks [P12] [P13] Housing Insecurity [TC](#)
- Water not working or unavailable [P12] [P13] Housing Insecurity [TC](#)
- Other, specify: \_\_\_\_\_ [P12] [P13] Housing Insecurity [TC](#)
- None
- Unknown

**Q3. Patient:** In the past year, how often did you worry that you would run out of food before you got money to buy more? **QM**

- Often [B8] [P12] [P14] Food Insecurity [TC](#)
- Sometimes [B8] [P12] [P14] Food Insecurity [TC](#)
- Never
- Unknown

**Q4. Patient:** In the past year, how often did the food run out and you did not have money to buy more? **QM**

- Often [B8] [B9] [P12] [P14] Food Insecurity [TC](#)
- Sometimes [B8] [B9] [P12] [P14] Food Insecurity [TC](#)
- Never



- Unknown

**Q5. Patient:** In the past year, have you missed medical appointments, work, or getting things needed for daily living because you did not have transportation? (Check all that apply.) **QM**

- Yes, it has kept me from medical appointments or getting medicines. [B10] [P12] [P15] Transportation  TC
- Yes, it has kept me from nonmedical activities, work, or getting things needed for daily living. [B10] [P12] [P15] Transportation  TC
- Other, specify: \_\_\_\_\_ [B10] [P12] [P15] Transportation  TC
- None
- Unknown

**Q6. Patient:** In the past year, has the electricity, water, gas, or oil company threatened to shut off services where you live?

- Yes, not shut off [P12] [P14]
- Yes, already shut off [B9] [P12] [P14]
- No
- Unknown

**Q7. Patient:** How often does anyone, including your family, physically hurt you?

- Never
- Rarely [P12] [P16] Intimate Partner Violence  TC
- Sometimes [P12] [P16] Intimate Partner Violence  TC
- Fairly often [P12] [P16] Intimate Partner Violence  TC
- Frequently [P12] [P16] Intimate Partner Violence  TC
- Unknown

**Q8. Patient:** How often does anyone, including your family, insult or talk down to you?

- Never
- Rarely [P12] [P16] Intimate Partner Violence  TC
- Sometimes [P12] [P16] Intimate Partner Violence  TC
- Fairly often [P12] [P16] Intimate Partner Violence  TC
- Frequently [P12] [P16] Intimate Partner Violence  TC
- Unknown

**Q9. Patient:** How often does anyone, including your family, threaten to harm you?

- Never
- Rarely [P12] [P16] Intimate Partner Violence  TC
- Sometimes [P12] [P16] Intimate Partner Violence  TC
- Fairly often [P12] [P16] Intimate Partner Violence  TC
- Frequently [P12] [P16] Intimate Partner Violence  TC
- Unknown

**Q10. Patient:** How often does anyone, including your family, scream or curse at you?

- Never
- Rarely [P12] [P16] Intimate Partner Violence  TC
- Sometimes [P12] [P16] Intimate Partner Violence  TC
- Fairly often [P12] [P16] Intimate Partner Violence  TC
- Frequently [P12] [P16] Intimate Partner Violence  TC
- Unknown

**Q11. Patient:** How hard is it for you to pay for the very basics, such as food, housing, medical care, and heating?

- Very hard [B9] [P12] [P14] Financial Status and Benefits  TC
- Somewhat hard [B9] [P12] [P14] Financial Status and Benefits  TC
- Not hard at all
- Unknown

**Q12. Patient:** Do you get the help you need with day-to-day activities, such as bathing, preparing meals, shopping, and managing finances?

- Yes

- No [P12] [P17] Caregiver Resources Evaluation [TC](#) Community Resources [TC](#)
- Unknown

**Q13. Patient:** Do you feel lonely or isolated from those around you?

- Yes [P12] [P17] Social Support [TC](#)
- No
- Unknown

**Q14. Patient:** Do you smoke, drink alcohol, or use drugs?

- Yes, specify: \_\_\_\_\_ Tobacco Use [TC](#) Alcohol Use [TC](#) Substance Use [TC](#)
- No
- Unknown Tobacco Use [TC](#) Alcohol Use [TC](#) Substance Use [TC](#)

## Care Planning

**Q1. Patient:** Have you started planning ahead for your healthcare?

- Yes [Q2]
- No [Q4] [P18]
- Unknown [P18]

**Q2. Patient:** Have you completed any advance care planning documents?

- Yes, specify: \_\_\_\_\_ [Q3]
- No [Q5] [P18]
- Unknown [Q4] [P18]

**Q3. Patient:** Who has a copy of your advance care planning documents? (Check all that apply.)

- Caregivers
- Clergy
- Doctors
- Family members
- Healthcare agent
- Hospitals
- Transportation provider
- Other, specify: \_\_\_\_\_
- None of these/Unknown [P18]

**Q4. Patient:** Are you interested in talking about advance care planning at this time?

- Yes [P18]
- No
- Unknown [P18]

**Q5. Patient:** Do you need help completing the documents?

- Yes [P18]
- No
- Unknown [P18]

**Q6. Patient:** What is your current healthcare coverage? (Check all that apply.)

- Private insurance, specify: \_\_\_\_\_ Benefits Evaluation [TC](#)
- COBRA Benefits Evaluation [TC](#)
- Coinsurance, specify: \_\_\_\_\_ Benefits Evaluation [TC](#)
- Disability, specify: \_\_\_\_\_ Benefits Evaluation [TC](#)
- Medicaid Benefits Evaluation [TC](#)
- Medicare (Part A, B, C, D), specify: \_\_\_\_\_
- Medigap (Medicare supplement Part F, G, K, L, M, N), specify: \_\_\_\_\_
- Medicare and Medicaid (dual eligible)
- Self-pay
- TRICARE Benefits Evaluation [TC](#)
- Veterans Affairs (VA) benefits Benefits Evaluation [TC](#)
- Workers' compensation Benefits Evaluation [TC](#)
- Other, specify: \_\_\_\_\_ Benefits Evaluation [TC](#)

- None [P19]
- Unknown [P19]

**Q7. Patient:** Does someone help with providing care? (Check all that apply.) **QM**

- Activities of daily living (eg, bathing, dressing, toileting) [Q8] [Q9] [Q10] [Q11] [Q12]
- Child care or parenting [Q8] [Q9] [Q10] [Q11] [Q12]
- Emotional support [Q8] [Q9] [Q10] [Q11] [Q12]
- Financial support [Q8] [Q9] [Q10] [Q11] [Q12]
- Health self-care (eg, taking medicines, checking vital signs) [Q8] [Q9] [Q10] [Q11] [Q12]
- Instrumental activities of daily living (IADL) or everyday tasks (eg, shopping, meal preparation, transportation) [Q8] [Q9] [Q10] [Q11] [Q12]
- Other, specify: \_\_\_\_\_ [Q8] [Q9] [Q10] [Q11] [Q12]
- No help needed
- Unknown [Q8] [Q9] [Q10] [Q11] [Q12]

**Q8. Patient:** Who helps you? (Check all that apply.) **QM**

- Spouse or partner
- Parent or adult child
- Extended family (eg, aunts, cousins)
- Friends
- Neighbors
- Paid caregivers
- Other, specify: \_\_\_\_\_
- No one
- Unknown
- Not applicable

**Q9. Patient:** How much time does the caregiver help? (Check all that apply.) **QM**

- 24 hours a day Caregiver Strain Evaluation [TC](#)
- 8 to 23 hours a day Caregiver Strain Evaluation [TC](#)
- 1 to 7 hours a day Caregiver Strain Evaluation [TC](#)
- 7 days a week Caregiver Strain Evaluation [TC](#)
- 4 to 6 days a week Caregiver Strain Evaluation [TC](#)
- 1 to 3 days a week Caregiver Strain Evaluation [TC](#)
- Other, specify: \_\_\_\_\_ Caregiver Strain Evaluation [TC](#)
- Unknown
- Not applicable

**Q10. Patient:** Does your caregiver have any problems? (Check all that apply.) **QM**

- Ability to provide care (eg, cannot do alone, has own health problems) [P20] Caregiver Strain Evaluation [TC](#)
- Abusive or abuse history [Q15] [P16] [P20] Caregiver Strain Evaluation [TC](#)
- Behavioral health diagnosis or history (eg, depression, substance misuse) [Q15] [P16] [P20] Caregiver Strain Evaluation [TC](#)
- Employment (eg, work schedule conflicts) [P20] Caregiver Strain Evaluation [TC](#)
- Family responsibilities (eg, caring for child limits caregiving support) [P20] Caregiver Strain Evaluation [TC](#)
- Information needed on how to provide care [P20] Caregiver Strain Evaluation [TC](#)
- Language barrier [P20] Caregiver Strain Evaluation [TC](#)
- Legal problems [Q15] [P16] [P20] Caregiver Strain Evaluation [TC](#)
- Self-interests missed (eg, hobbies, social events) [P20] Caregiver Strain Evaluation [TC](#)
- Stress management ineffective (eg, overwhelmed) [P20] Caregiver Strain Evaluation [TC](#)
- Other, specify: \_\_\_\_\_ [P20] Caregiver Strain Evaluation [TC](#)
- No problems identified
- Unknown [P20] Caregiver Strain Evaluation [TC](#)
- Not applicable

**Q11. Patient:** How is your caregiver doing with your care? **QM**

- Meets all needs

- Meets most needs [B11] [P20] Caregiver Strain Evaluation [TC](#)
- Meets about 1/2 of needs [B11] [P16] [P20] Caregiver Strain Evaluation [TC](#)
- Does not meet most needs [B11] [P16] [P20] Caregiver Strain Evaluation [TC](#)
- Does not meet needs [B11] [P16] [P20] Caregiver Strain Evaluation [TC](#)
- Unknown [P20]
- Not applicable

**Q12. Patient:** Is there anyone else who can help with your care? (Check all that apply.) **QM**

- Spouse or partner
- Parent or adult child
- Extended family (eg, aunts, cousins)
- Friends
- Neighbors
- Paid caregivers
- Other, specify: \_\_\_\_\_
- No one [B11] [P20] Community Resources [TC](#)
- Unknown [P20]

**Q13. Patient:** Do you feel safe where you live? **QM**

- Yes
- No, because: \_\_\_\_\_ [B11] [P16] [P20]
- Unknown [P16]

**Q14. Case Manager:** Is there adequate, safe caregiver support? **QM**

- Yes
- No [B11] [P20]
- Unknown [P16]

**Q15. Case Manager:** Is abuse, exploitation, or neglect suspected? **QM**

- Yes [P16]
- No
- Unknown [P16]

**Q16. Patient:** Do you get any support from community resources (such as Meals on Wheels, support groups, or help from a social worker)?

- Yes, specify: \_\_\_\_\_
- No
- Unknown

**Q17. Patient:** Do you have any self-care needs that are not being met? (Check all that apply.)

- Affording medicine or supplies [B12] [P21] Financial Status and Benefits [TC](#)
- Caregiver help needed [B12] [P21] Caregiver Resources Evaluation [TC](#)
- Daily care activities (eg, bathing, dressing, feeding self) [B12] [P21]
- Home safety problems [B12] [P21] Safety [TC](#)
- Managing meals (eg, cooking, shopping) [B12] [P21]
- Shopping activities (eg, grocery, pharmacy) [B12] [P21]
- Social support [B12] [P21] Social Support [TC](#)
- Transportation for care [B12] [P21] Transportation [TC](#)
- Trouble understanding how to do self-care [B12] [P21] Health Literacy [TC](#)
- Other, specify: \_\_\_\_\_ [B12] [P21]
- No
- Unknown

## Quality Measures

- Disease Management Quality Measures:
  - Information provided from this question may be requested by various accrediting organizations.

## Plan of Care

## Barriers

- B1.** Health literacy barrier
- B2.** Linguistic barrier
- B3.** Cultural barriers
- B4.** Hearing impairment
- B5.** Vision impairment
- B6.** Cognitive impairment present
- B7.** Housing inadequate
- B8.** Food insufficient
- B9.** Financial barrier
- B10.** Transportation inadequate
- B11.** Limited or no caregiver resources
- B12.** Limited community resources

## Problems

### P1. Language barrier present

- **Goal:** Language barrier resolved 
- **Interventions:**
  - **ASSIST:** Identifying translation options and using the most practical and appropriate one(2)
  - **COORDINATE:** Call with patient and translation service and obtaining consent from patient to use translator(3)(4)
  - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
  - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
  - **EDUCATE:** Ways to overcome language barriers(2)(5):
    - Speak slowly in normal tone.
    - Discuss one topic at a time.
    - Repeat and summarize often.
    - Use names instead of pronouns.
    - Use short, simple words and sentences.
    - Use interpretation or translation services.(6)
  - **COORDINATE:** Communication to providers to offer education materials in native language, whenever possible(6)(7)
  - **COORDINATE:** Communication between care team members and care plan reconciliation(8)(9)(10)
  - **COORDINATE:** Follow-up contact to evaluate language barrier status, referral status, and need for further assistance
  - **SEND:** What you need to know - Chronic condition management  (Spanish)

### P2. Cultural barriers to healthcare present

- **Goal:** Cultural needs and practices incorporated into plan of care(6)(11)(12) 
- **Interventions:**
  - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
  - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
  - **ASSIST:** Identifying ways that cultural needs and practices can be incorporated into patient's plan of care(13)
  - **ASSIST:** Appointment scheduling with provider to evaluate incorporation of cultural preferences into self-care plan
  - **EDUCATE:** Considerations of cultural practices on self-care, if appropriate
  - **COORDINATE:** Community services referral, as needed[A]
  - **COORDINATE:** Cultural practices incorporated into care planning across providers
  - **COORDINATE:** Home care referral, if indicated
  - **COORDINATE:** Social worker referral, as needed(14)
  - **COORDINATE:** Communication between care team members and care plan reconciliation(10)
  - **COORDINATE:** Follow-up contact to evaluate cultural practices and self-care plan, status of referrals, and need for further assistance(15)(16)

### P3. Hearing evaluation needed

- **Goal:** Hearing evaluated and self-care understood 
- **Interventions:**
  - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
  - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
  - **ASSIST:** Obtaining consent for communication with patient's caregiver for program continuation

- **ASSIST:** Appointment scheduling for hearing evaluation
- **EDUCATE:** When to get help for hearing problems (eg, infection, hearing problems get worse, social isolation, or depression)(17)(18)
- **EDUCATE:** Hearing loss implications (eg, safety risks)(17)
- **COORDINATE:** Audiologist referral, if indicated(17)
- **COORDINATE:** Home care referral, if indicated
- **COORDINATE:** Care team meeting, if needed(10)(19)
- **COORDINATE:** Communication between care team members and care plan reconciliation(8)(9)(10)
- **COORDINATE:** Follow-up contact to evaluate hearing status, referral status, and need for further assistance(15)(16)
- **SEND:** What you need to know - Chronic condition management  (Spanish)

#### P4. Hearing impairment present

- **Goal:** Hearing maximized(20)
  - **Interventions:**
    - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
    - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
    - **ASSIST:** Obtaining consent for communication with patient's caregiver for program continuation
    - **ASSIST:** Appointment scheduling for evaluation of hearing impairment(17)
    - **ASSIST:** Informing patient's providers of hearing loss and preferred alternative communication method
    - **ASSIST:** Identifying resources for hearing aids, phone lines, and other needed equipment and training
    - **ASSIST:** Utilizing a sign language interpreter trained for medical settings who can assist during medical appointments, if appropriate
    - **ASSIST:** Obtaining instructions in writing as much as possible(21)
    - **EDUCATE:** Ways to work around hearing loss(17)(21):
      - Hearing aid or assistive device
      - Writing or drawing pictures
      - Reading lips
      - Technical aids (eg, TTY phone service or closed-captioning)
      - Gestures or sign language
    - **EDUCATE:** Hearing aids, benefit, use, and care, if appropriate(22)
    - **EDUCATE:** Hearing loss implications (eg, safety risks)(22)
    - **EDUCATE:** Ways to prevent further hearing problems(17)(22):
      - Avoid places with loud persistent noises.
      - Keep environmental noises down when trying to listen.
      - Manage ear problems (eg, infection) quickly.
    - **EDUCATE:** Ways caregivers can help with communication(17)(23)(24):
      - Devote full attention when speaking or listening to patient, and avoid distracting environments (eg, those with competing noise).
      - Face patient when speaking, talk slowly, and do not cover mouth.
      - Ask patient to repeat back what they heard.
      - Avoid jumping from topic to topic, and speak in simple sentences.
      - Provide other ways to communicate (eg, offering paper for writing).
      - Promote use of hearing aids, as appropriate.(25)
    - **EDUCATE:** When to get help for hearing problems (eg, infection, hearing problems get worse, social isolation, or depression)(17)(25)
    - **EDUCATE: Resource:** Hearing Loss Association of America: [www.hearingloss.org](http://www.hearingloss.org)
    - **COORDINATE:** Otolaryngologist referral, if indicated(18)
    - **COORDINATE:** Audiologist referral, if indicated(17)
    - **COORDINATE:** Rehabilitation therapy referral, if indicated(26)
    - **COORDINATE:** Home care referral, if indicated
    - **COORDINATE:** Care team meeting, if needed(10)(19)
    - **COORDINATE:** Communication between care team members and care plan reconciliation(8)(9)(10)
    - **COORDINATE:** Follow-up contact to evaluate hearing status, referral status, and need for further assistance(15)(16)
    - **SEND:** What you need to know - Chronic condition management  (Spanish)

#### P5. Vision impairment present

- **Goal:** Vision maximized(20)(27)
  - **Interventions:**
    - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up

- **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
- **ASSIST:** Obtaining consent for communication with patient's caregiver for program continuation
- **ASSIST:** Appointment scheduling for evaluation of visual impairment<sup>N</sup>
- **ASSIST:** Informing patient's providers of vision loss and patient's preference for communication
- **ASSIST:** Identifying resources for visual aids (glasses, contact lenses) and other needed equipment and training
- **ASSIST:** Obtaining written materials in braille, if indicated
- **EDUCATE:** Ways to work around vision loss(28):
  - Visual aids or assistive devices (glasses, contact lenses)
  - Alternative ways to perform activity
  - Voice options for electronics
  - Sighted companion
  - Cane and mobility training
- **EDUCATE:** Implications of vision impairment(28):
  - Interference with ADL and increased risk for falls and other injuries
  - Impact on psychological, emotional, and social well-being
- **EDUCATE:** Ways to prevent further vision problems:
  - Keep eyes healthy (nutrition support, washing hands before eye care, wearing protective eyewear).
  - Importance of evaluation for changes in vision
  - Manage eye problems (eg, infection) quickly.
- **EDUCATE:** When to get help for vision problems (eg, infection, vision problems get worse, social isolation, or depression)(25)(28)
- **EDUCATE: Resource:** American Foundation for the Blind: [www.afb.org](http://www.afb.org)
- **EDUCATE:** Routine eye examinations important
- **COORDINATE:** Optometrist referral, if indicated
- **COORDINATE:** Ophthalmologist referral, if indicated
- **COORDINATE:** Rehabilitation therapy referral, if indicated(28)
- **COORDINATE:** Medical equipment and supplies, if indicated
- **COORDINATE:** Home care referral, if indicated
- **COORDINATE:** Care team meeting, if needed(10)(19)
- **COORDINATE:** Communication between care team members and care plan reconciliation(8)(9)(10)
- **COORDINATE:** Follow-up contact to evaluate vision status, referral status, and need for further assistance(15)(16)
- **SEND:** What you need to know - Chronic condition management  (Spanish)

#### P6. Vision evaluation needed

- **Goal:** Vision evaluated and self-care understood(27)<sup>N</sup>
  - **Interventions:**
    - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
    - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
    - **ASSIST:** Obtaining consent for communication with patient's caregiver for program continuation
    - **ASSIST:** Appointment scheduling with provider for visual evaluation
    - **EDUCATE:** When to get help for vision problems (eg, infection, vision problems get worse, social isolation, or depression)(25)(28)
    - **EDUCATE:** Vision loss implications (eg, safety risks)(20)
    - **COORDINATE:** Optometrist referral, if indicated
    - **COORDINATE:** Ophthalmologist referral, if indicated
    - **COORDINATE:** Home care referral, if indicated
    - **COORDINATE:** Follow-up contact to evaluate vision status, referral status, and need for further assistance(15)(16)
    - **SEND:** What you need to know - Chronic condition management  (Spanish)

#### P7. Medication information needed

- **Goal:** Medication information understood
  - **Interventions:**
    - **ASSIST:** Creation of current and reconciled medication list
    - **ASSIST:** Identifying activities that promote medication adherence
    - **ASSIST:** Identifying medication support persons (eg, family, caregiver) to assist patient with managing medication(29)
    - **EDUCATE: Reason for taking medication(30):**
      - Expected effects if the medication is working, and expected time for effects to occur
      - How long medication will likely need to be taken
      - What happens if medication is not taken as prescribed



- **EDUCATE:** Importance of carrying a list of medications at all times, particularly when visiting healthcare providers
- **EDUCATE:** Information on all medications(29)(30)(31):
  - Names and purpose of all medications
  - Amount to take (as prescribed), such as number of pills or amount of liquid, and how to take, such as by mouth or injection
  - When and how to take the medication
  - Common side effects and serious side effects
  - When and how to get help for problems with medication
  - What to do if doses are missed
  - How to dispose of unused or expired medication(32)
- **COORDINATE:** Communication between patient and pharmacy (for obtaining prescriptions, setting up home medication delivery, review for drug-drug interactions)[N](#)
- **COORDINATE:** Home care referral, if indicated
- **COORDINATE:** Communication between care team members and reconciled medication list.(30) See Medication Reconciliation Tool [SR](#) for more information.
- **COORDINATE:** Follow-up to evaluate adherence or problems with medication, provide additional education and support, and identify need for further assistance(15)(16)
- **SEND:** Managing your medicines (Illustrated)  (Spanish)

#### P8. Medication refills needed

- **Goal:** Medications refilled
  - **Interventions:**
    - **RECONCILE:** Medication list. See Medication Reconciliation Tool [SR](#) for more information.[N](#)
    - **ASSIST:** Appointment scheduling with prescribing provider for medication evaluation and refills
    - **ASSIST:** Obtaining refills in lieu of an appointment
    - **ASSIST:** Identifying resources to pay for medications, if needed
    - **ASSIST:** Working with pharmacy for filling prescriptions (eg, changing pills to liquids, identifying financial support, delivery)
    - **EDUCATE:** Refill medications prior to running out (1 to 2 weeks before).
    - **EDUCATE:** Pharmacy assistance programs, such as:
      - Centers for Medicare and Medicaid Services (CMS): [www.medicare.gov/drug-coverage-part-d](http://www.medicare.gov/drug-coverage-part-d)
      - Medicine Assistance Tool: <https://medicineassistancetool.org>
      - NeedyMeds: [www.needymeds.org](http://www.needymeds.org)
      - Individual company websites
    - **COORDINATE:** Communication between patient and pharmacy (for obtaining prescriptions, setting up home medication delivery, review for drug-drug interactions)
    - **COORDINATE:** Prescription refill support (eg, pharmacy programs) referral
    - **COORDINATE:** Follow-up to evaluate continued need for refills, referral status, and need for further assistance(16)(33)
    - **SEND:** Managing your medicines (Illustrated)  (Spanish)

#### P9. Complementary, alternative, or integrative therapies evaluated for effect on treatment plan and outcomes

- **Goal:** Complementary, alternative, or integrative therapies will support positive treatment outcomes.(34)(35)(36)
  - **Interventions:**
    - **ASSIST:** Identifying alternative treatments patient may be using
    - **ASSIST:** Appointment scheduling with provider to evaluate participation in complementary, alternative, or integrative therapy with treatment plan
    - **EDUCATE:** Importance of talking to care provider before starting any complementary, alternative, or integrative therapy
    - **EDUCATE:** Some complementary, alternative, or integrative therapies can interfere with treatment and outcomes.
    - **EDUCATE:** Known benefits and risks with participation in complementary, alternative, or integrative therapy
    - **EDUCATE:** Select complementary, alternative, or integrative therapy providers who are appropriately licensed (if applicable), trained, and experienced.
    - **EDUCATE:** Resource for more information: National Center for Complementary and Integrative Health: [www.nccih.nih.gov](http://www.nccih.nih.gov)
    - **COORDINATE:** Social worker referral, as needed
    - **COORDINATE:** Communication between care team members regarding use of complementary, alternative, or integrative therapies
    - **COORDINATE:** Follow-up contact to evaluate treatment problems, referral status, and need for further assistance(16)(33)

#### P10. Vaccinations not up to date, increasing infection risk

- **Goal:** Vaccinations up to date(37)(38)
  - **Interventions:**



- **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
- **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
- **ASSIST:** Appointment scheduling
- **ASSIST:** Identifying vaccines patient may need that are not contraindicated based on clinical status
- **EDUCATE:** Purpose and benefits of vaccinations
- **EDUCATE:** Recommended immunizations and timing of administration:
  - Centers for Disease Control and Prevention's vaccination schedule: [www.cdc.gov/vaccines](http://www.cdc.gov/vaccines)(39)
  - Vaccine schedules may need to be altered based on medications; check with the specialist provider for information.(40)
  - Possible side effects or complications associated with getting vaccinated
- **EDUCATE:** Influenza vaccination annually(41):
  - Side effects are mild and last 1 or 2 days.
  - Common side effects with shot are sore, red, or swollen area where shot was given; fever; and aches.
  - Common side effects with nasal spray are mild and last 1 or 2 days; they include runny nose, headache, sore throat, and cough.
  - Allergic reaction rare and is usually caused by a chicken egg allergy
- **EDUCATE:** Pneumococcal vaccination potential side effects:
  - Side effects are mild and last 1 or 2 days.
  - Common side effects include a sore, red, or swollen area where shot was given; fever; and muscle aches.
  - Serious side effects are rare.
- **EDUCATE:** COVID-19 vaccination potential side effects(39):
  - Side effects are mild and last 1 or 2 days.
  - Common side effects include a sore, red, or swollen area where shot was given; chills; fatigue; and aches.
  - Serious side effects are rare.
- **COORDINATE:** Community services referral, as needed
- **COORDINATE:** Vaccinations
- **COORDINATE:** Follow-up to evaluate vaccination understanding, referral status, and need for further assistance(15)(16)
- **SEND:** Coronavirus disease 2019 (COVID-19) vaccines 🇪🇸 (Spanish)
- **SEND:** Flu vaccine 🇪🇸 (Spanish)
- **SEND:** Pneumonia vaccine for adults 🇪🇸 (Spanish)
- **SEND:** Hepatitis A vaccine 🇪🇸 (Spanish)
- **SEND:** Hepatitis B vaccine 🇪🇸 (Spanish)

#### P11. Cognitive impairment present

- **Goal:** Cognitive and behavioral symptoms addressed and managed(42)(43)
  - **Interventions:**
    - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
    - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
    - **ASSIST:** Support system (eg, family, friends) identification, enlistment, and education
    - **ASSIST:** Appointment scheduling for evaluation of new or worsening change in behavioral or cognitive function, and the development of a self-care plan
    - **ASSIST:** Creation of a self-care plan between patient and provider (eg, goals, management, treating symptoms, getting help)
    - **ASSIST:** Communication between multiple providers(44)
    - **EDUCATE:** Cognition information(45):
      - Cognition is ability to learn, solve problems, remember, and appropriately use information.
      - Problems or deficits are part of aging process, but specific disorders may be treatable (eg, delirium, dementia, depression).
      - Cognition deficits may be progressive.
      - When and how to get help if cognition problems progress to point caregiver cannot provide care
    - **EDUCATE:** Ways to help manage cognitive changes(45)(46):
      - Agitation management (eg, simplify tasks and instructions, keep routine consistent)
      - Maintain a calm environment.
      - Set small goals every day (eg, get out of bed at a certain time, finish one chore or project, or do something that brings pleasure, such as listening to music).
      - Structured activities (eg, exercise therapy, pet therapy, social contact)
      - Focus on process instead of results.

- Break down all tasks into simple steps.
- ADL/IADL scheduled at times during the day when better able to perform or participate
- Encourage participation in ADL and IADL, but do not force.
- Organizational strategies and problem-solving tasks, including note-taking devices, cueing, and using memory books
- Mentally stimulating activities participation (eg, reading, puzzles)
- Orientation aids (eg, calendars, clocks, and alarms)
- **EDUCATE:** Medication side effects for cognitive and behavioral deficits (eg, antipsychotics, tricyclic antidepressants, SSRI antidepressants, dementia drugs)
- **EDUCATE:** Cognitive deficits may be progressive, and how to get help if they progress to point caregiver cannot provide care.
- **EDUCATE: When to get emergency help:**
  - Suicidal or homicidal actions or plans(47)
  - Psychotic behavior (eg, threatening)
  - Heart attack (eg, persistent chest pain; pain radiating to the arms, shoulders, neck, back, and/or jaw; cool clammy skin)(48)
  - Stroke (eg, face drooping, arm weakness, speech problems, bad headache, vision changes, loss of balance or coordination)(49)
- **EDUCATE: When to call the provider(46):**
  - Depression (eg, feeling sad, hopeless, overwhelmed)(47)
  - Hallucinations or delusions increase.
  - Cognitive changes (eg, thinking problems, forgetfulness worsens)
  - Insomnia
  - Self-care changes (eg, ADL or IADL problems)
- **COORDINATE:** Behavioral health services referral, if indicated
- **COORDINATE:** Home care referral, if indicated
- **COORDINATE:** Neurologist referral, if indicated
- **COORDINATE:** Psychiatrist referral, if indicated
- **COORDINATE:** Community services referral, as needed
- **COORDINATE:** Social worker referral, as needed(14)
- **COORDINATE:** Support group referral, as needed
- **COORDINATE:** Rehabilitation therapy referral, if indicated[B](43)
- **COORDINATE:** Caregiver health service referral, as needed
- **COORDINATE:** Care team meeting, if needed(10)(19)
- **COORDINATE:** Communication between care team members and care plan reconciliation(8)(9)(10)
- **COORDINATE:** Follow-up evaluation for problems due to cognitive impairment, referral status, and need for further assistance(15)(16)
- **SEND:** Caregiver tips 📄 (Spanish)
- **SEND:** Helping people who cannot think clearly 📄 (Spanish)
- **SEND:** Safety in the home - Memory loss 📄 (Spanish)
- **SEND:** Help with sleeping or insomnia 📄 (Spanish)
- **SEND:** Tips for good mental health 📄 (Spanish)
- **SEND:** Recognizing the signs of depression 📄 (Spanish)

#### P12. Social determinant of health need present

- **Goal:** Social determinant of health needs assessed and addressed(50)(51)(52)(53) **NN**
  - **Interventions:**
    - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
    - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
    - **EDUCATE:** Assessment of social determinants of health is routinely completed on every person.(54)(55)(56)
      - The purpose is to identify resources that could be helpful. Information is still valuable to the program even if the person does not have any needs in a particular domain.
      - Availability of resources and interventions varies by region.
    - **EDUCATE: Resources:**
      - Benefit Finder: [www.benefits.gov](http://www.benefits.gov)
      - Findhelp: [www.findhelp.org](http://www.findhelp.org)
      - United Way: [www.211.org](http://www.211.org)
    - **COORDINATE:** Increased intervals of follow-up with preferred method of communication (eg, phone, text, email)
    - **COORDINATE:** Social worker referral, as needed(14)

- **COORDINATE:** Community services referral, as needed
- **COORDINATE:** Follow-up contact to evaluate social needs status, referral status, and need for further assistance(15)(16)

**P13. Housing inadequate, unstable, or unsafe**

- **Goal:** Housing modified or changed so it is adequate, stable, and safe(57)(NN)
- **Interventions:**
  - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
  - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
  - **ASSIST:** Identifying housing options
  - **ASSIST:** Completing documentation for housing assistance, if needed
  - **ASSIST:** Appointment scheduling with provider if housing problems negatively affecting self-care
  - **COORDINATE:** Social worker referral, as needed(14)
  - **COORDINATE:** Community services referral, as needed(58)(59)
  - **COORDINATE:** Follow-up contact to evaluate housing status, referral status, and need for further assistance(15)(16)

**P14. Financial issues present**

- **Goal:** Financial issues addressed(57)(60)(61)(62)(NN)
- **Interventions:**
  - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
  - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
  - **ASSIST:** Identifying financial support programs(63)
  - **ASSIST:** Contacting and completing paperwork for financial support programs
  - **ASSIST:** Appointment scheduling
  - **EDUCATE:** Healthcare benefits and out-of-pocket expenses
  - **EDUCATE: Resources:**
    - Administration for Children & Families: [www.acf.hhs.gov](http://www.acf.hhs.gov)
    - Centers for Medicare and Medicaid Services (CMS): [www.medicare.gov/drug-coverage-part-d](http://www.medicare.gov/drug-coverage-part-d)
    - Food and Nutrition Service (FNS) Nutrition Programs: [www.fns.usda.gov/programs](http://www.fns.usda.gov/programs)
    - Medicine Assistance Tool: <https://medicineassistancetool.org>
    - NeedyMeds. Find help with the cost of medicine: [www.needymeds.org](http://www.needymeds.org)
  - **COORDINATE:** Financial assistance referral; examples include:
    - Area Agency on Aging
    - Disease advocacy
    - Financial counseling or assistance
    - Food banks or other sources for nutritional support and financial assistance
    - Government agencies
    - Legal aid
    - Long-term disability application
    - Medicaid application
    - Medicare application
    - Pharmacy assistance programs
    - Short-term disability application
    - Supplemental Security Income (SSI) or disability benefits application
    - Veterans Affairs (VA) benefits application
  - **COORDINATE:** Care team meeting, if needed(10)(19)
  - **COORDINATE:** Communication between care team members and care plan reconciliation(8)(9)(10)
  - **COORDINATE:** Follow-up for evaluation of financial issues, referral status, and need for further assistance(15)(16)

**P15. Transportation inadequate for patient's needs**

- **Goal:** Transportation needs adequately met(57)(60)(64)(N)
- **Interventions:**
  - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
  - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
  - **ASSIST:** Identifying transportation resources and accessing as needed(11)
  - **ASSIST:** Completing application forms for disability transportation, if needed
  - **EDUCATE:** Type of transportation resources and how to access them

- **EDUCATE:** When and how to get help if transportation continues to be a problem
- **COORDINATE:** Social worker referral, as needed(14)
- **COORDINATE:** Community services referral, as needed(65)
- **COORDINATE:** Communication between care team members on how patient is transported to and from service visits(8)(9)(10)
- **COORDINATE:** Follow-up contact to evaluate transportation, referral status, and need for further assistance(15)(16)

**P16. Personal safety risk present**

- **Goal:** Safety issues identified and addressed(57)
  - **Interventions:**
    - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
    - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
    - **ASSIST:** Contact with legal authorities, as appropriate(66)
    - **EDUCATE:** When and how to get help in unsafe situations
    - **EDUCATE: Resources:**
      - National Domestic Violence Hotline: [www.thehotline.org](http://www.thehotline.org); 1-800-799-7233, or call, text, or live chat 88788
      - National Suicide Prevention Lifeline: [www.988lifeline.org](http://www.988lifeline.org); 1-800-273-8255, or call, text, or live chat 988
    - **COORDINATE:** Protective services or legal authorities, as needed
    - **COORDINATE:** Social worker referral, as needed(14)
    - **COORDINATE:** Home care referral, if indicated(66)
    - **COORDINATE:** Behavioral health services referral, if indicated
    - **COORDINATE:** Follow-up contact to evaluate safety, referral status, and need for further assistance(15)(16)

**P17. Social support needs present**

- **Goal:** Social support needs identified and addressed
  - **Interventions:**
    - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
    - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
    - **ASSIST:** Identifying caregivers and community resources
    - **ASSIST:** Accessing caregivers and community resources, if needed
    - **ASSIST:** Completing application forms for community resources, if needed
    - **EDUCATE:** When and how to get help if social support continues to be a problem
    - **COORDINATE:** Social worker referral, as needed(14)
    - **COORDINATE:** Home care referral, if indicated(66)
    - **COORDINATE:** Follow-up contact to evaluate social support, referral status, and need for further assistance(15)(16)

**P18. Advance care planning incomplete**

- **Goal:** Advance care planning information provided and decision made to complete advance directives or not(67)(68)**N**
  - **Interventions:**
    - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
    - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
    - **ASSIST:** Accessing information about advance care planning that is appropriate within state legal requirements(67)
    - **ASSIST:** Appointment scheduling
    - **ASSIST:** Identifying values and beliefs that influence end-of-life decisions(69)
    - **ASSIST:** Identifying a healthcare proxy to make decisions when patient not able to
    - **ASSIST:** Accessing and obtaining written physician orders for life-sustaining treatment that include the level of medical intervention to be delivered in the event of an emergency (comfort only, limited treatment, or full treatment) and use of artificial hydration and nutrition(67)
    - **ASSIST:** Distributing advance care planning forms in collaboration with patient, family, and caregiver
    - **ASSIST:** Using community resources for assistance with advance care planning forms, in coordination with patient, family, and caregiver
    - **ASSIST:** Discussion on goals, values, and types and locations of care(69)
    - **EDUCATE:** Advance care planning (ACP) information(28)(67):
      - Documents patient's preferences/instructions for using, withholding, or withdrawing life-sustaining treatments and end-of-life care
      - Treatment goals and postmortem care can be part of advance directives (ie, curative vs palliative care, burial vs cremation).

- Appoints medical decision maker for when/if the patient is unable to make own decisions
- Changes permitted, as preferences may change (eg, as patient ages or viewpoint changes)
- Types of ACP documents include living wills, healthcare/medical power of attorney (HCPOA or MPOA)
- Benefits of ACP: aligns patient's preferences with care plans and reduces treatment decision-making stress for caregivers
- States have different laws on what are considered acceptable/legal advance directives.
- **EDUCATE:** Medical care power of attorney, or durable power of attorney, appoints someone (called a healthcare agent or a "proxy") to make healthcare decisions when the patient is unable to.[C]
- **EDUCATE:** Living will communicates the patient's wishes on key interventions and treatment decisions(70)(71):
  - Resuscitation procedures (or not)(67)
  - Mechanical respirations
  - Chemotherapy or radiation therapy
  - Dialysis
  - Nutrition care (eg, supplemental feeding, fluids)(67)
  - Simple diagnostic tests
  - Organ donation
  - Pain management
  - Blood transfusions
  - ICD and pacemakers(72)
  - Intentional deep sedation
- **EDUCATE:** Reasons for distribution of advance care planning forms
- **EDUCATE:** Distribution list for advance care planning forms: providers, hospitals, healthcare agent, family members, caregivers, clergy
- **EDUCATE: Resources:**
  - Advance Care Planning - National Institute on Aging: [www.nia.nih.gov](http://www.nia.nih.gov)
  - Five Wishes - Aging with Dignity: [www.agingwithdignity.org](http://www.agingwithdignity.org)
  - Physician Orders for Life-Sustaining Treatment Paradigm program: [www.polst.org](http://www.polst.org)
  - Put It In Writing - American Hospital Association: [www.aha.org](http://www.aha.org)
- **COORDINATE:** Communication among patient, family, caregiver, and providers about value of advance care planning
- **COORDINATE:** Social worker referral, as needed(14)
- **COORDINATE:** Care team meeting, if needed(10)(19)
- **COORDINATE:** Communication between care team members and advance directive status
- **COORDINATE:** Follow-up to evaluate advance directive, referral status, and need for further assistance(15)(16)
- **SEND:** Advance care planning  (Spanish)
- **SEND:** Advance directive ("living will") form
- **SEND:** Healthcare power of attorney form

**P19.** Potential complication: financial issues present

- **Goal:** Financial resources identified and sufficient to continue care(61)(62) 
- **Interventions:**
  - **ASSIST:** Identifying healthcare benefits coverage from payers
  - **ASSIST:** Identifying availability of and eligibility for alternative financial resources
  - **ASSIST:** Communicating information on patient's healthcare benefits coverage with treatment team
  - **ASSIST:** Appointment scheduling and completing paperwork to support applications for financial support
  - **EDUCATE:** Healthcare benefits and out-of-pocket expenses
  - **EDUCATE:** Resources for financial assistance:
    - Administration for Children & Families: [www.acf.hhs.gov](http://www.acf.hhs.gov)
    - Centers for Medicare and Medicaid Services (CMS): [www.medicare.gov/drug-coverage-part-d](http://www.medicare.gov/drug-coverage-part-d)
    - Medicine Assistance Tool: <https://medicineassistancetool.org>
    - NeedyMeds. Find help with the cost of medicine: [www.needymeds.org](http://www.needymeds.org)
    - Individual pharmaceutical company websites
  - **COORDINATE:** Financial assistance referral; examples include:
    - Area Agency on Aging
    - Disease advocacy
    - Financial counseling or assistance
    - Food banks or other sources for nutritional support and financial assistance
    - Government agencies
    - Legal aid
    - Long-term disability application
    - Medicaid application
    - Medicare application
    - Pharmacy assistance programs

- Short-term disability application
- Supplemental Security Income (SSI) or disability benefits application
- Veterans Affairs (VA) benefits application
- **COORDINATE:** Social worker referral, as needed(73)
- **COORDINATE:** Follow-up for evaluation of referral status and need for further assistance(9)(15)
- **Goal:** Financial issues addressed(62)
  - **Interventions:**
    - **RECONCILE:** Benefits from multiple sources, if needed
    - **ASSIST:** Obtaining preauthorization from health plan provider, if appropriate
    - **EDUCATE:** Source and type of financial support, how to access, and if follow-up needed
    - **COORDINATE:** Follow-up for evaluation of use of support programs or ongoing need for further assistance(9)(15)

## P20. Caregiver assistance needed

- **Goal:** Caregiver assistance identified and provided(24)(63)
  - **Interventions:**
    - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
    - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
    - **ASSIST:** Identifying additional caregivers to offer support
    - **ASSIST:** Identifying caregiver alternatives (eg, adult day care, community support groups, respite programs)
    - **EDUCATE:** Importance of patient performing own self-care as much as they are able
    - **EDUCATE:** Caregiver support(74):
      - Identification of other caregivers
      - Consideration of adult day care or respite program
      - Self-care needs (eg, medical, behavior health, social support)
    - **EDUCATE:** When and where to get help if caregiver support discontinued or insufficient
    - **COORDINATE:** Home care referral, if indicated(75)
    - **COORDINATE:** Respite care referral, as needed
    - **COORDINATE:** Caregiver health service referral, as needed
    - **COORDINATE:** Community services referral, as needed
    - **COORDINATE:** Support group referral, as needed
    - **COORDINATE:** Social worker referral, as needed(14)
    - **COORDINATE:** Communication between care team members and monitoring caregiver status(9)(10)
    - **COORDINATE:** Follow-up with patient and caregiver for evaluation of burden, status of referrals, identification of problems, and need for further assistance(15)(16)
    - **SEND:** Caregiver tips 📄 (Spanish)
    - **SEND:** Choosing a place to live 📄 (Spanish)
    - **SEND:** Hospice care 📄 (Spanish)
- **Goal:** Caregiver education addressed(63)(76)
  - **Interventions:**
    - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
    - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
    - **ASSIST:** Support system (eg, family, friends) identification, enlistment, and education(63)
    - **EDUCATE:** Information to help caregivers:
      - Long-term caregiving is a chronic stressor and may be associated with excessive caregiver burden and adverse impact on physical and emotional health.
      - Know and act on signs of depression in self and patient.
      - Disease process and management strategies
      - Expected benefits and side effects of medications
      - Importance of maintaining social contacts and support, and asking for help, as appropriate
      - Importance of prioritizing tasks to free up time to spend on their own needs. Encourage self-care.
      - Use and value of adult day care or respite programs
      - Other housing placement options
      - Stress management (eg, relaxation and coping techniques)
      - Seek help from religious affiliation, if applicable.
      - Seek healthcare for own medical conditions, as appropriate.
    - **COORDINATE:** Educational program referral for caregiver(9)
    - **COORDINATE:** Community services referral, as needed
    - **COORDINATE:** Follow-up on caregiver needs, referral status, and need for further assistance(15)(16)



**P21. Community resources needed**

- **Goal:** Community resources identified and coordinated(24)(77)(78) 
- **Interventions:**
  - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
  - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
  - **ASSIST:** Appointment scheduling with provider for patient at risk of self-care problems or to support referrals to community services (eg, completing medical forms for transportation service)
  - **ASSIST:** Identifying available community resources and contacting for support(79)
  - **EDUCATE:** Types of community resources available(80)
  - **EDUCATE: Resources:**
    - Benefit Finder: [www.benefits.gov](http://www.benefits.gov)
    - Findhelp: [www.findhelp.org](http://www.findhelp.org)
    - United Way: [www.211.org](http://www.211.org)
  - **EDUCATE:** Role of community services to support self-care
  - **COORDINATE:** Home care referral, if indicated
  - **COORDINATE:** Community services referral, as needed
  - **COORDINATE:** Support group referral, as needed
  - **COORDINATE:** Social worker referral, as needed(14)
  - **COORDINATE:** Follow-up to evaluate community resources, referral status, and need for further assistance(15)(16)

## Evidence Summary

**Quality standards:** Standards indicate that a needs assessment should be completed for every patient to identify and resolve potential communication barriers, including vision impairments, hearing impairments, and languages spoken other than English. Programs should have a process for following up with patients on the status of referrals.(81)

**Quality standards:** Standards indicate that program content should address psychosocial issues (eg, cultural practices) that may impede self-management. Programs should have a process for following up with patients on the status of referrals.(81)

**National guideline:** The U.S. Preventive Services Task Force recognizes that age-related sensorineural hearing loss can adversely affect quality of life and functional abilities and is associated with increased risk of falls, hospitalizations, social isolation, and cognitive decline; however, there is insufficient evidence to assess the balance of benefits and harms of screening patients without symptoms of hearing loss.(82)

**Evidence:** In a study of 99 adults (age 62 to 82 years), self-reported overall quality of life was significantly improved at 18 months after hearing aid fitting.(83)

**Evidence:** Multiple studies conclude that hearing loss contributes to social isolation and mood disorders and is associated with increased rates of cognitive impairment.(17)(84)(85)

**National guideline:** The U.S. Preventive Services Task Force recognizes that vision screening of older adults is important for the detection of age-related vision changes (eg, cataracts, refractive errors, adjustment to light intensity changes, distinguishing color, glare tolerance); however, there is insufficient evidence that vision screening in primary care for patients without symptoms of vision problems results in improved functional outcomes (eg, fewer falls).(86)

**Outcomes:** Several studies support the role of the pharmacist in medication reconciliation to identify and resolve pharmacy-related problems and improve outcomes (eg, prevention of medication-related adverse events, discontinuation of inappropriate medication).(87)

**Quality standards:** Standards indicate that medications should be documented and any identified discrepancies communicated to the appropriate provider.(81)

**Quality standards:** Standards indicate that program content should address psychosocial issues that would impede self-management. Programs should have a process for following up with patients on the status of referrals.(81)

**Evidence:** A review of 4 independent studies with 14,749 low-income adults indicated that participants who were less healthy, had lower incomes, and had higher levels of stress and depressive symptoms were consistently more likely to have multiple social needs than those who were healthier and had higher incomes, lower stress, and fewer depressive symptoms. Women with children were less likely to need food, transportation, or money for necessities. Older, less healthy adults needed food, transportation, and money for necessities and unexpected expenses.(88)

**Quality standards:** Standards indicate that program content should address psychosocial issues (eg, inadequate or unsafe housing) that would impede self-management. Programs should have a process for following up with patients on the status of referrals.(81)

**Evidence:** A retrospective review of 22,324 medical records determined that 3 unstable housing circumstances (behind on rent in the past 12 months, 2 or more moves in the past 12 months, and a history of homelessness in the child's lifetime) are associated with adverse caregiver and child health status, developmental risk, and other household hardships.(89)

**Quality standards:** Standards indicate that program content should address psychosocial issues (eg, financial barriers) that would impede self-management. Programs should have a process for following up with patients on the status of referrals.(81)

**Evidence:** A retrospective study of 7959 nationally representative US children determined that household food insecurity is associated with higher emergency department use, higher school absenteeism, and lower access to care.(90)

**Quality standards:** Standards indicate that program content should address psychosocial issues (eg, transportation) that would impede self-management. Programs should have a process for following up with patients on the status of referrals.(81)

**Quality standards:** Standards indicate that program content should include a condition treatment plan and information on external resources; in addition, patient-provider communication should be encouraged (eg, with information on what to do before, during, and after a provider visit). Programs should have a process for following up with patients on the status of referrals.(81)

**Quality standards:** Standards indicate that programs should have a process for identifying barriers to patients meeting goals or adhering to the case management plan and for following up with patients on the status of referrals.(81)

**Quality standards:** Standards indicate that program content should address referrals to external resources that may support self-management. Programs should have a process for following up with patients on the status of referrals.(81)

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## Footnotes

[A] Community services may include cultural organizations that offer support.(12) [ A in Context Link 1 ]

[B] Many driver rehabilitation programs provide therapy and behind-the-wheel assessments. Therapy assessment covers motor, cognitive, perception, and reaction times. The behind-the-wheel assessment covers fitness to drive.(91) [ B in Context Link 1 ]

[C] In the United States, most states have legislation that delineates decision-making authority for patients without advance directives (a default surrogate), but there is a lot of variation in prioritizing, appointing, and the process for challenging default surrogates. In states that have a surrogate hierarchy, the priority is usually: spouse, adult child, parent.(67) [ C in Context Link 1 ]

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